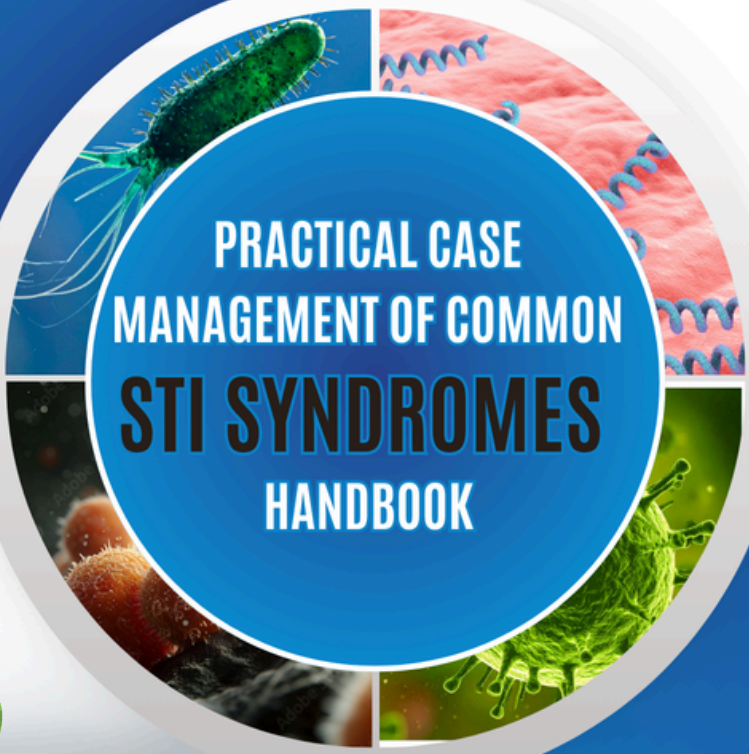
MINISTRY OF HEALTH & WELLNESS

NATIONAL HIV/STI/TB PROGRAMME

2026



# PRACTICAL CASE MANAGEMENT OF COMMON STI SYNDROMES HANDBOOK

SPECIALLY ADAPTED FOR USE IN PRIMARY HEALTH CARE CENTRES

52-60 Grenada Crescent, Kingston 5

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# CHECKLIST FOR MEDICAL HISTORY TAKING

**A. Demographics**

1. National Identification number / medical registration number
2. Name
3. Age
4. Date of Birth
5. Sex at birth
6. Gender identity
7. Telephone no. (home / work / if any)
8. Address (indicate if permanent and current residence; indicate if transient, and include landmarks / street number)
9. Next of kin / emergency contact (name, relationship, phone number)
10. Marital status (single / married / separated / divorced / widowed; include duration if applicable)
11. Living arrangements (lives alone / with family / partner / other)
12. Occupation (state type of work, employment status, duration, and if unemployed, reason)
13. Date first seen

**B. Present Medical History**

1. Reason for present visit
2. Symptoms (state duration / severity / course)
3. Treatment taken (state result / sample drug)

**C. Gynaecologic/Obstetric History**

1. **Menstrual history:** Age at menarche, cycle pattern, duration, flow, LMP, and recent changes to pattern and associated symptoms
2. **Gynaecologic history:** Last pap smear, past pelvic conditions, gynaecologic surgeries, infertility, or abnormal bleeding/discharge, douching practices.
3. **Obstetric history:** Gravidity, parity, outcomes of pregnancies, and complications.

**D. Past Medical History**

1. Chronic illnesses or immunocompromising conditions (indicate if any steroid use)
2. Surgical History (indicate if any genito-anal surgeries)
3. Medication history and drug allergy (state recent antibiotic use within the last 2 weeks)
4. Vaccination History – HBV, HPV (state type and dose completion)
5. Contraception (state the type and frequency of use)
6. History of substance abuse
7. History of blood or blood product transfusions

**E. Family History**

1. Significant familial conditions (indicate congenital infections, reproductive disorders, infertility)
2. Health status of parents & siblings (include cause of death, if applicable)

# CHECKLIST FOR SEXUAL HISTORY TAKING

**A. Presenting Complaint**

1. Symptoms (discharge, ulcers, rash, itching, pain, dysuria, etc.)
2. Duration and recurrence
3. Recent treatment or self-medication

**B. Sexual Exposure History (last 3 months)**

1. Currently sexually active
2. Date(s) of last sexual exposure
3. Relationship to last sexual partner(s) — spouse, new / regular partner, or other
4. Sexual practices during last exposure (penetrative or non-penetrative – fisting, fingering, nudging, dipping, masturbation - alone or shared)
5. Anatomical site(s) of exposure

**C. Sexual Exposure History (greater than 3 months)**

1. Age at sexual debut
2. Number of sexual partners in the past 3 months
3. Number of lifetime sexual partners
4. Type(s) of sexual contact – vaginal, anal, oral, other (specify)
5. Sex of partners – male, female, both
6. Vaginal intercourse during menses
7. Nature of relationships – regular, casual, transactional, anonymous
8. PrEP (use and type)

**D. Condom and Contraceptive Use**

1. Frequency of condom use (always / sometimes / never)
2. Correct use (start to finish of act)
3. Condom breakage or slippage
4. Condom use with whom (new / regular, causal partners)
5. Condom use at last sexual encounter
6. Use of other contraceptive methods (type / consistency)

**E. Partner(s) Sexual History**

1. Partner(s) with STI symptoms or diagnosis
2. Contact with new /regular partner(s) in the last 3 months
3. Sex of partners - male, female, both
4. Known HIV positive
5. Partner(s) on PrEP or ART
6. Aware of STI infection
7. History of substance abuse

**F. Other Personal Risk Factors:**

1. HIV infection
2. History of GBV/IPV

## Table 1: Coding for STIs and Other Conditions

| **New Cases of Syphilis** | |
|---|---|
| Primary | A1 |
| Secondary | A2 |
| Early latent (in the first year of infection) | A3 |
| Cardiovascular | A4 |
| Neurological | A5 |
| All other late and late latent stages | A6 |
| Congenital (aged under 2 years) | A7 |
| Congenital (aged 2 years and over) | A8 |
| **New cases of gonorrhoea** | |
| Post-pubertal infection | B1 |
| Pre-pubertal infection | B2 |
| Ophthalmia Neonatorum | B3 |
| **New Cases of other STIs** | |
| Chancroid | C1 |
| Lymphogranuloma venereum | C2 |
| Granuloma inguinale | C3 |
| Non-gonococcal urethritis/cervicitis | C4 |
| Non-specific genital infection with arthritis | C5 |

## Table 1: Coding for STIs and Other Conditions

| **New Cases of Other STIs continued** | |
|---|---|
| Trichomoniasis | C6 |
| Candidiasis | C7 |
| Herpes simplex virus | C10 |
| Warts (condylomata acuminata/Human Papilloma Virus) | C11 |
| Molluscum contagiosum | C12 |
| HIV infection | C13 |
| Hepatitis B virus | C14 |
| HTLV-1 | C15 |
| **New cases of other conditions** | |
| Other treponemal disease e.g., yaws | D1 |
| Other conditions requiring treatment in the centre e.g., MPox | D2 |
| Other conditions not requiring treatment in the centre | D3 |

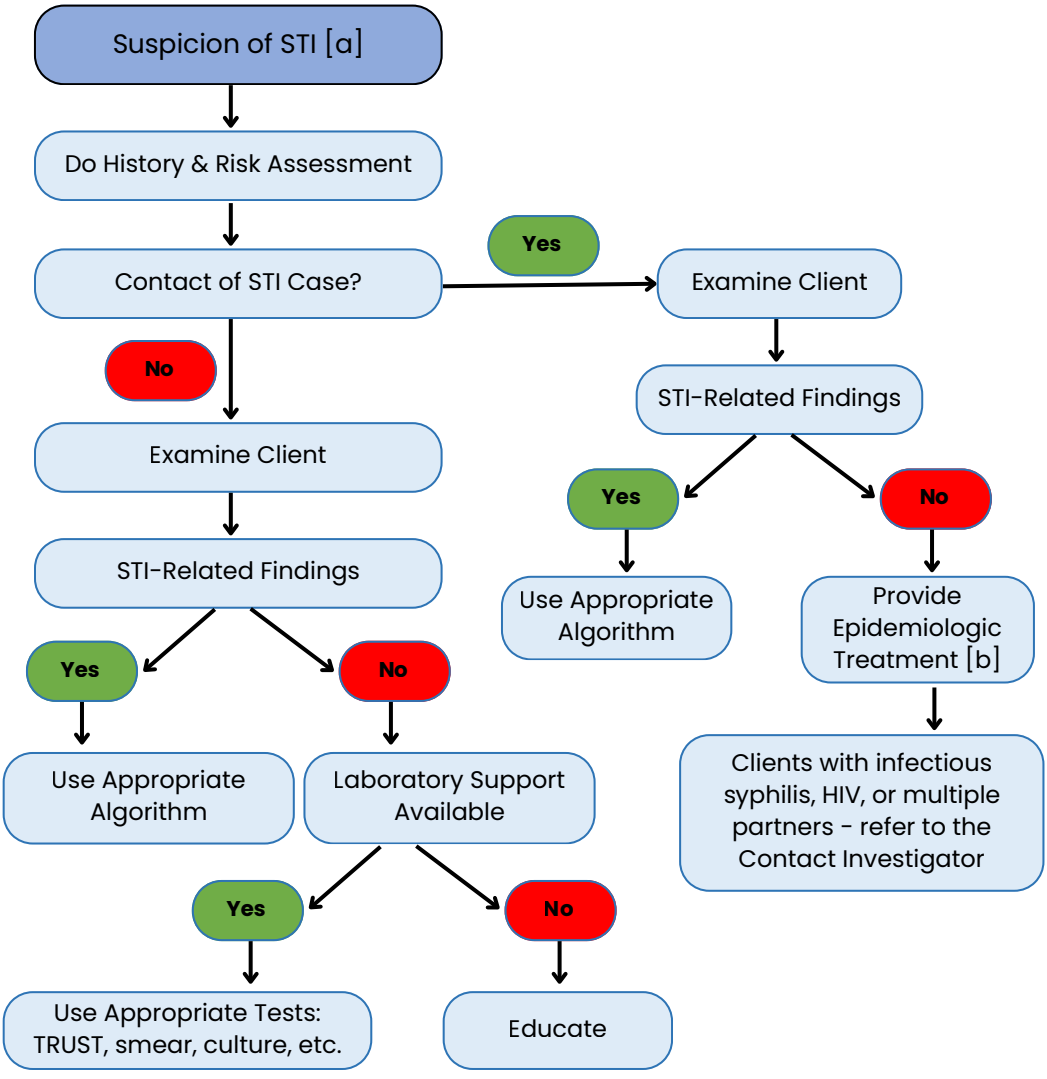
## Table 2: Common Signs and Symptoms of STIs in Men

| Symptoms (Reported by Patient) | Signs (Observed on Examination) |
|---|---|
| Urethral discharge, leaking, or dripping from penis | Urethral discharge (mucoid, purulent, or blood-stained) |
| Dysuria (burning sensation or pain on urination) | Erythema or swelling of the urethral meatus |
| Sores or “water bumps” on genitals | Genital ulcers or vesicles |
| Lower abdominal or pelvic discomfort | Lower abdominal tenderness |
| Scrotal pain or heaviness | Scrotal swelling or tenderness |
| Lump, bump, or growth on genitals | Genital warts |
| Rash | Rash on palms, soles, or body |
| Rectal pain, discharge, or bleeding | Perianal or rectal discharge or lesions |
| Swollen glands | Inguinal lymphadenopathy |
| Sore throat or sores in the mouth | Pharyngeal exudates or ulcers to the oral mucosa |

## Table 3: Common Signs and Symptoms of STIs in Women

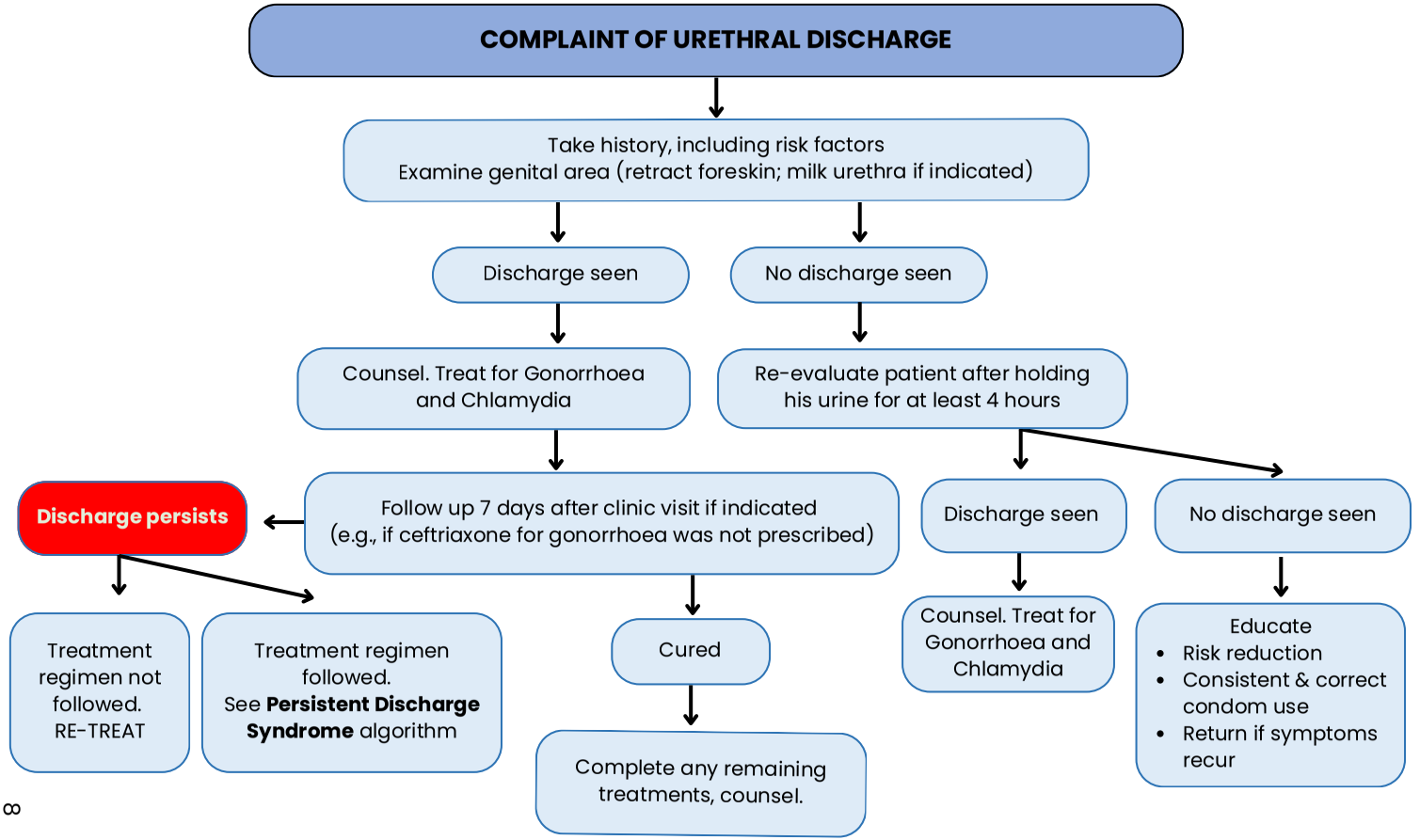
| Symptoms (Reported by Patient) | Signs (Observed on Examination) |
|---|---|
| Vaginal discharge (change in colour, odour, or amount) | Abnormal vaginal discharge (amount, odour, consistency) |
| Itching or irritation of the vulva or vagina | Vulval or vaginal erythema and edema |
| Sores or “water bumps” on genitals | Ulcers or vesicles on the genitalia |
| Dyspareunia (pain during intercourse), lower abdominal or pelvic pain | Cervical discharge or friability, cervical motion tenderness +/- adnexal tenderness or mass |
| Dysuria (burning or painful urination) | Erythema or swelling of the urethral meatus |
| Lump, bump, or growth on genitals | Genital warts |
| Rash | Rash involving palms, soles, or body |
| Vaginal odour (fishy, foul, or musty smell) | Abnormal vaginal discharge (amount, odour, consistency) |
| Abnormal vaginal bleeding or spotting | Cervical friability, cervical mass |
| Swollen glands | Inguinal lymphadenopathy |
| Painful defecation, rectal discomfort, or discharge | Perianal, rectal discharge, or lesions |
| Sore throat or sores in the mouth | Pharyngeal exudates or ulcers to the oral mucosa |

**Figure 1: Algorithm for STI Screening**




**Note to Figure 1:**
a. If the patient has signs, use the appropriate algorithm
b. Appropriate explanation should be given as to why (s)he needs treatment

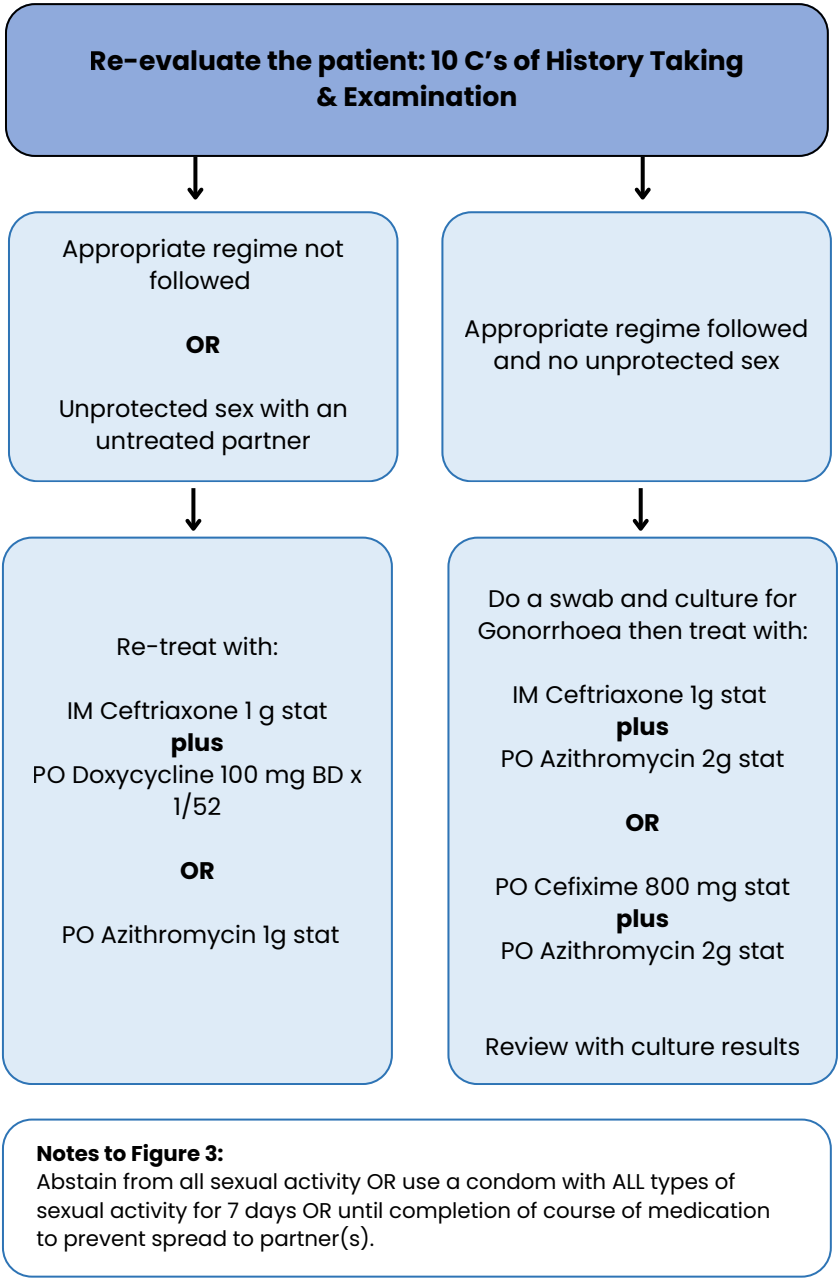
## Figure 2: Algorithm for Management of Urethral Discharge



**Note to Figure 2:**

- Early morning bead of pus; urinary “thread” in first urine passed; WBCs > 4/HPF on Gram-stained smear - suggests chlamydial/non-gonococcal urethritis
- If urethral discharge persists and the client is >35 years of age, refer to a urologist or hospital for further evaluation, as non-STI causes of urethritis may be present.

**Figure 3: Algorithm for Management of Persistent Urethral Discharge**




**Notes to Figure 3:**
Abstain from all sexual activity OR use a condom with ALL types of sexual activity for 7 days OR until completion of course of medication to prevent spread to partner(s).

## Table 4: Treatment of Urethral Discharge (Gonococcal and Chlamydial Urethritis)

<table>
<tr>
<td rowspan="2">Recommended treatment:<br><br>Ceftriaxone 1g, intramuscularly, single dose<br>PLUS<br>Doxycycline 100 mg, orally, twice daily for 7 days</td>
<td><u>Effective substitutes for Ceftriaxone include:</u><br>Cefixime 800mg, orally, single dose*<br>OR<br>Ciprofloxacin 500mg, orally, single dose</td>
<td rowspan="2">Avoid doxycycline in pregnancy, lactation, and persons 16 years and under.</td>
</tr>
<tr>
<td><u>Effective substitutes for Doxycycline include:</u><br>Azithromycin 1g single dose orally<br>OR<br>Erythromycin 500 mg, orally, 4 times a day for 7 days<br>OR<br>Ofloxacin 200–400 mg, orally, twice a day for 7 days</td>
</tr>
<tr>
<td colspan="2">If symptoms persist or reoccur after 7 days (without evidence of reinfection)<br><br>1. Do a swab and test for gonorrhoea and chlamydia (e.g., Nucleic Acid Amplification (NAAT), Culture and Sensitivity tests)<br>2. Re-treat with:<br><br>Ceftriaxone 1g, intramuscularly, single dose<br>PLUS<br>Azithromycin 2g, orally<br>PLUS<br>**Metronidazole 2g, orally, single dose (OR 500mg, orally, twice daily for 7 days)</td>
<td>Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals.</td>
</tr>
</table>

*If using the alternative option for gonococcal infection **(cefixime 800mg)** add azithromycin 2g to cover chlamydial infection.

**In settings with limited laboratory capacity, add metronidazole to the management of urethritis to cover possible *Trichomonas vaginalis* infection.

**Figure 4: Algorithm for Management of Testicular Pain**

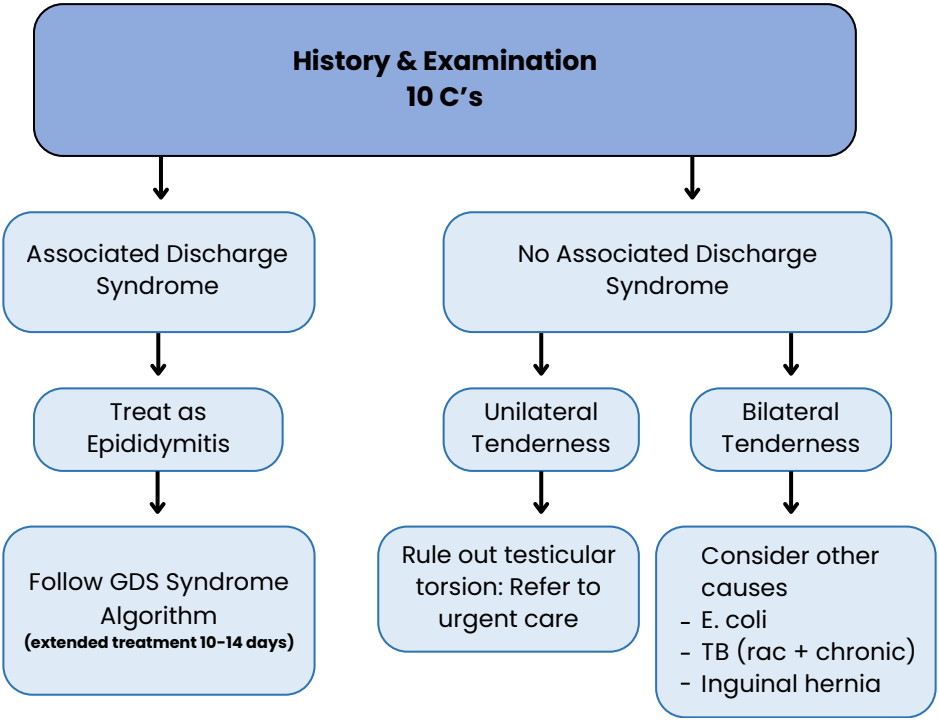
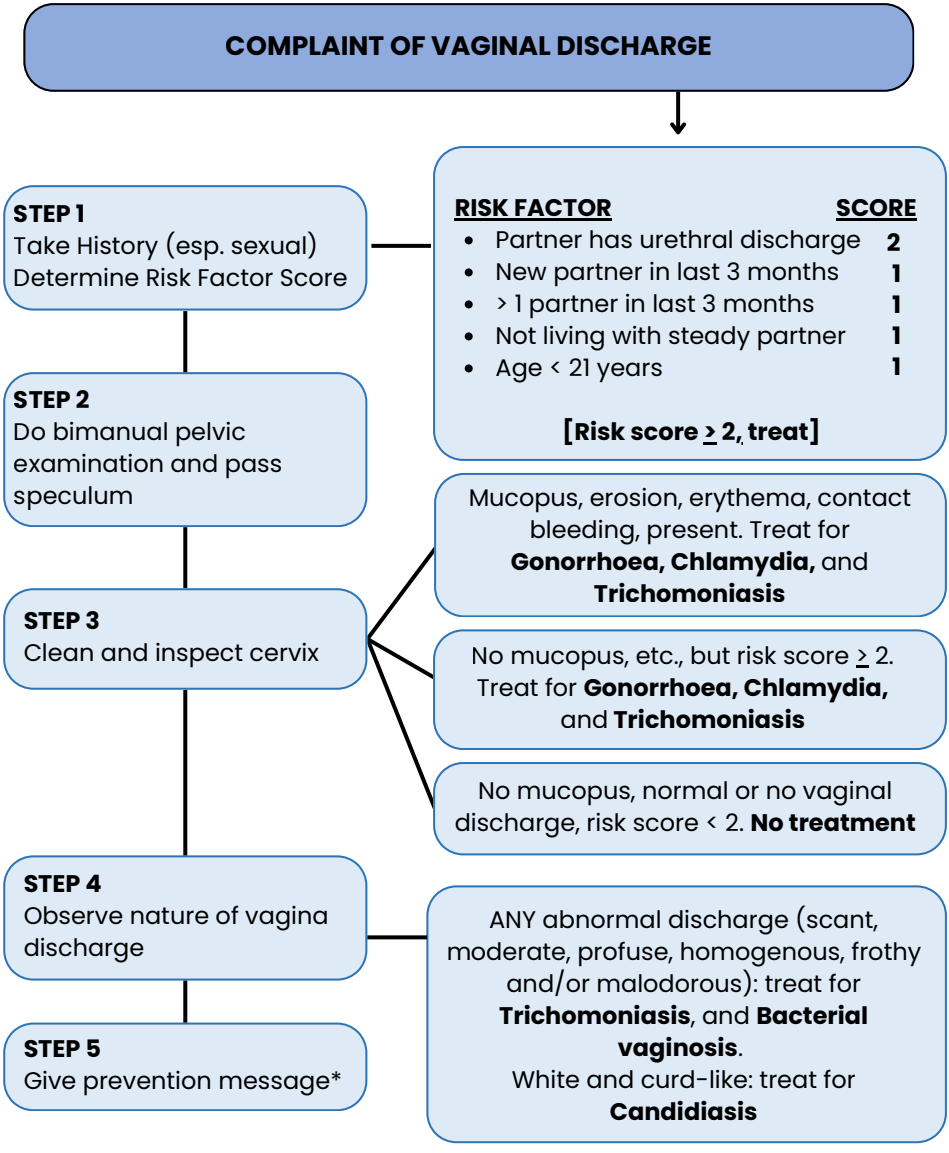


**Figure 5: Algorithm for Management of Cervico-vaginal Discharge**




**Notes to Figure 5:**

- Comply with medication; condoms; contacts, counsel re risk reduction.
- **Screen all patients for HIV/Syphilis.**
- **Do pap smears and refer persistent cases of discharge to the gynaecologist.**

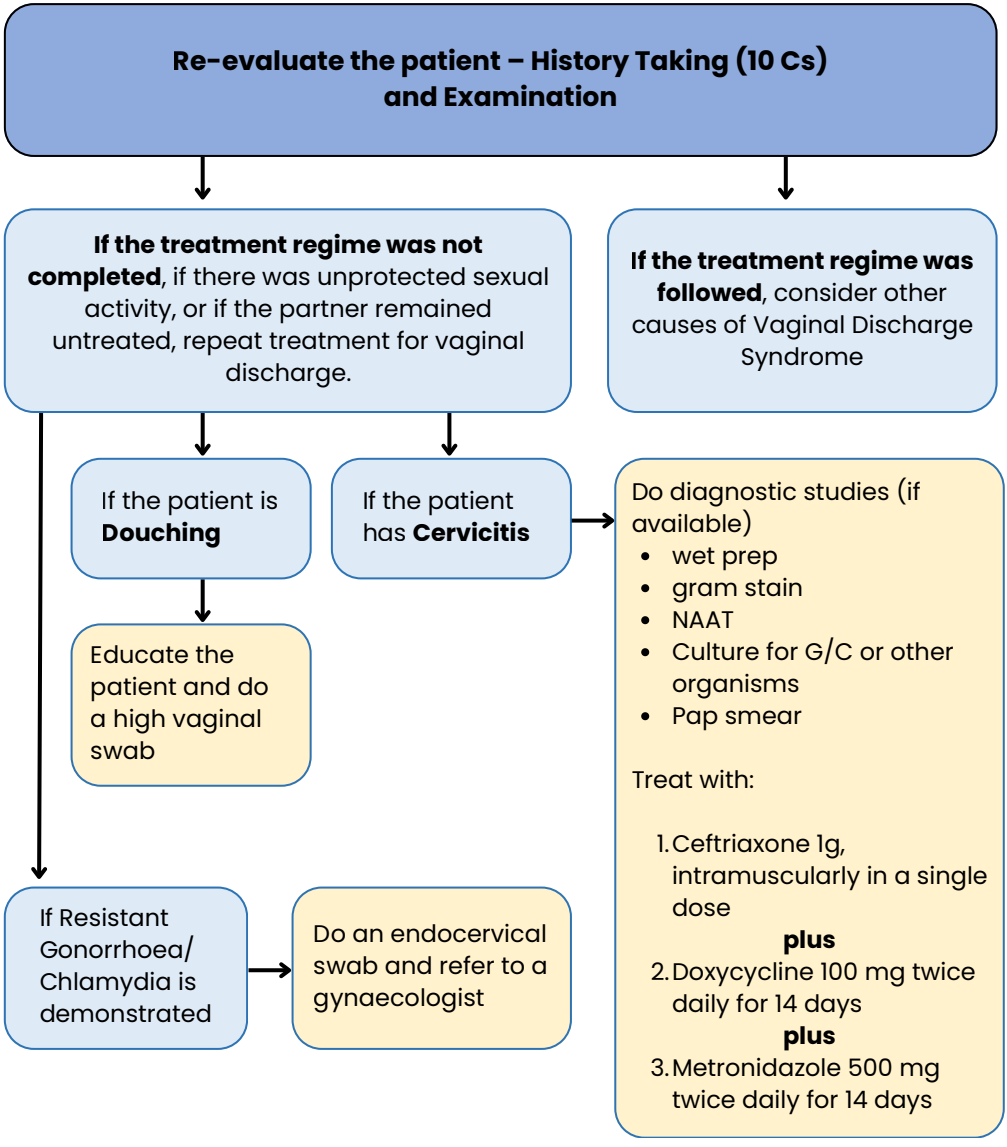
## Table 5: Treatment of Cervico-vaginal Discharge (Gonococcal and Chlamydial Urethritis)

<table>
<tr>
<td rowspan="2">Recommended treatment:<br><br>Ceftriaxone 1g, intramuscularly, single dose<br>PLUS<br>Doxycycline 100 mg, orally, twice daily for 7 days<br>PLUS<br>**Metronidazole 500mg, orally, twice daily for 7 days<br><br>If cervicitis is present, increase the duration of Doxycycline and Metronidazole to 14 days.</td>
<td><u>Effective substitutes for Ceftriaxone include:</u><br>Cefixime 800mg, orally, single dose*<br>OR<br>Ciprofloxacin 500mg, orally, single dose</td>
<td rowspan="2"></td>
</tr>
<tr>
<td><u>Effective substitutes for Doxycycline include:</u><br>Azithromycin 1g single dose orally<br>OR<br>Erythromycin 500 mg, orally, 4 times a day for 7 days<br>OR<br>Ofloxacin 200–400 mg, orally, twice a day for 7 days</td>
<td>Avoid doxycycline in pregnancy, lactation, and persons 16 years and under.</td>
</tr>
<tr>
<td colspan="2">If symptoms persist or reoccur after 7 days (without evidence of reinfection)<br><br>1. Do a swab and test for gonorrhoea and chlamydia (e.g., Nucleic Acid Amplification (NAAT), Culture and Sensitivity tests)<br>2. Re-treat with:<br>Ceftriaxone 1g, intramuscularly, single dose<br>PLUS<br>Azithromycin 2g, orally<br>PLUS<br>Metronidazole 2g, orally, single dose (OR 500mg, orally, twice daily for 7 days)</td>
<td>Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals.</td>
</tr>
</table>

*If using the alternative option for gonococcal infection **(cefixime 800mg)** add azithromycin 2g to cover chlamydial infection.

**In settings with limited laboratory capacity, add metronidazole to the management of cervico-vaginal discharge to cover possible *Trichomonas vaginalis* infection.

**Figure 6: Algorithm for Management of Persistent Vaginal Discharge**

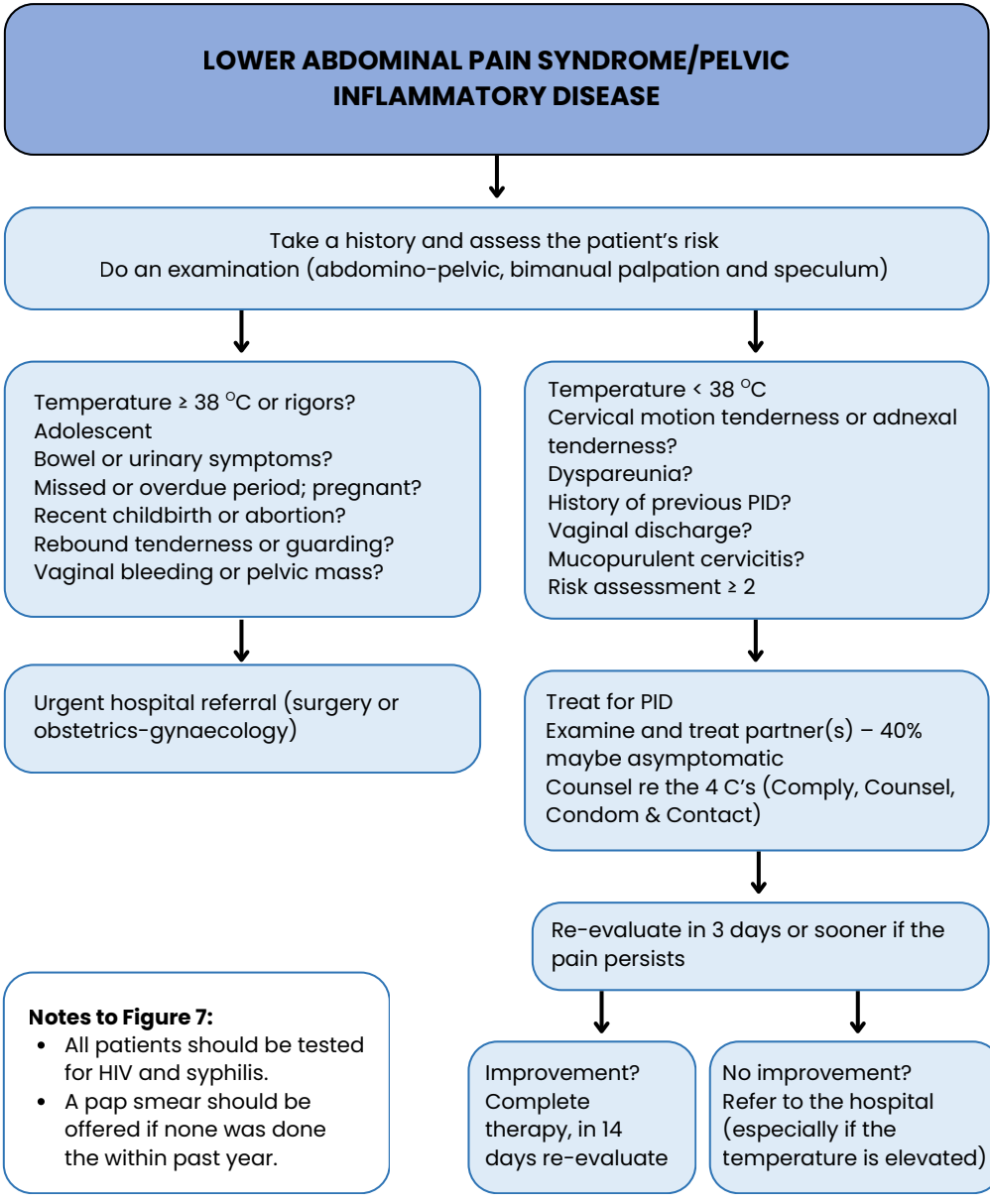



**Note to Figure 6:**
NAAT: Nucleic Acid Amplification Test

## Table 6: Treatment of Vaginal Discharge Syndrome

| Aetiology | Recommended Treatment | Effective Substitute | Notes |
|---|---|---|---|
| Bacterial Vaginosis | Metronidazole 400 mg **or** 500 mg, orally, twice daily for 7 days | • Clindamycin 300 mg, orally, twice daily for 7 days **or**<br>• Metronidazole 2 grams, orally, single dose **or**<br>• Metronidazole 200 mg or 250 mg, orally, 3 times a day for 7 days **or**<br>• Metronidazole gel 0.75%, one full applicator (5 grams), intravaginally, twice a day for 7 days **or**<br>• Clindamycin 300 mg, orally, twice daily for 7 days | Metronidazole should be avoided during the **first trimester** of pregnancy unless the potential benefits outweigh the associated risks. |
| Trichomoniasis | • Metronidazole 2 grams in a single dose **or**<br>• Metronidazole 400 mg or 500mg orally, twice daily for 7 days | Effective substitutes for Metronidazole to treat Trichomonas include:<br>• Tinidazole 2 grams orally in a single dose **or**<br>• Tinidazole 500 mg orally, twice daily for 5 days **or**<br><br>Other substitutes:<br>• Metronidazole 200 mg or 250 mg, orally, 3 times a day for 7 days **or**<br>• Metronidazole gel 0.75%, one full applicator (5 grams), intravaginally, twice a day for 7 days | |
| Candidiasis | • Clotrimazole vaginal tablet, 100 mg, inserted at night for 7 nights **or**<br>• Miconazole vaginal pessaries, 200 mg inserted at night for 3 nights | Effective substitutes include:<br>• Fluconazole 150 mg (or 200mg), orally, single dose **or**<br>• Nystatin pessaries 200,000 units, inserted at night for 7 night<br><br>Additional substitutes include:<br>• Miconazole 200 mg vaginal pessaries inserted once daily for 3 days **or**<br>• Clotrimazole vaginal tablet 100 mg inserted at night for 7 days | |

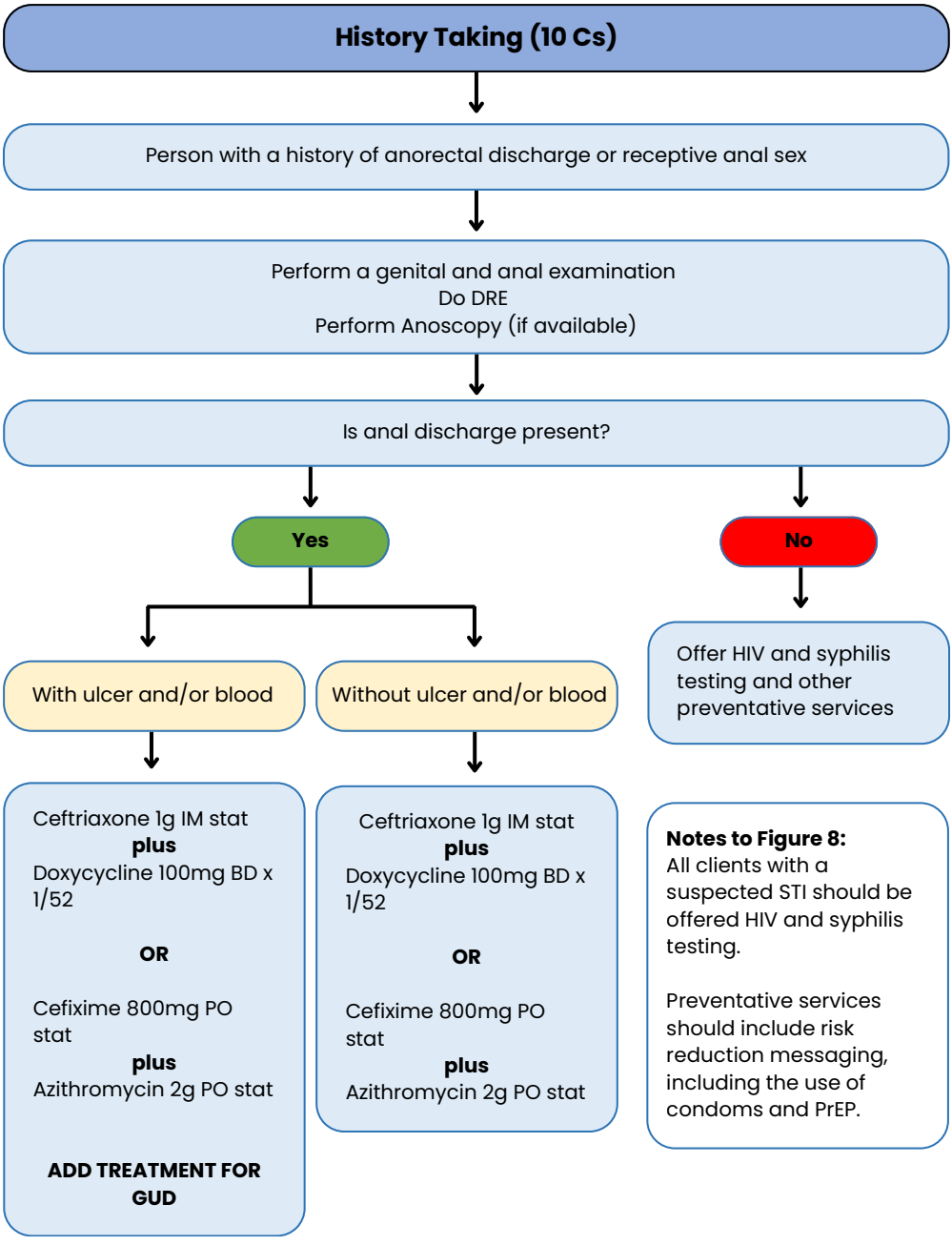
**Figure 7: Algorithm for Management of Lower Abdominal Pain/PID**




**Notes to Figure 7:**

- All patients should be tested for HIV and syphilis.
- A pap smear should be offered if none was done the within past year.

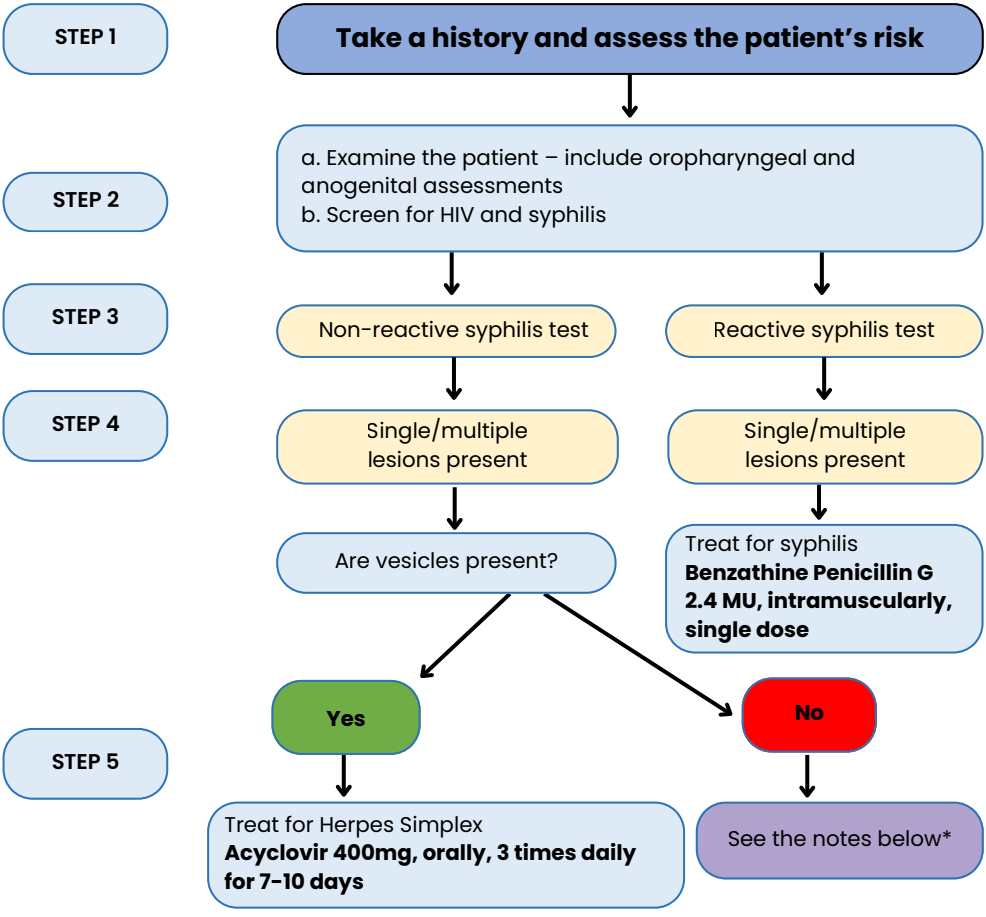
## Figure 8: Algorithm for Management of Anal Discharge



## Table 7: Treatment of Anal Discharge Syndrome

| Recommended Treatment | Effective Substitutes | Notes |
|---|---|---|
| **Ceftriaxone 1g, intramuscularly, single dose**<br>**PLUS**<br>**Doxycycline 100 mg, orally, twice daily for 7 days** | Cefixime 800mg, orally, single dose<br>**PLUS**<br>Azithromycin 2g, orally | Azithromycin may cause gastrointestinal side effects – consider giving Azithromycin 1g at 12-hour intervals. |

## Figure 9: Algorithm for Management of Genital Ulcer Disease (GUD)




**Notes for Figure 9:**
Encourage normal saline sitz baths.
Review the ulcer weekly for improvement.
If the ulcer persists or worsens after treatment, consider other etiologies and refer to a STI specialists or dermatologist or an infectious disease specialist for further investigation.

**Step 1:** Take a comprehensive sexual history and assess risk. Ask about ulcers or recurrent lesions. *Note: Syphilitic chancres heal without scarring.*
**Step 2: Perform a comprehensive examination – include oropharyngeal and anogenital assessments**; screen for HIV and syphilis.
**Step 3:** Review screening test results.
**Step 4:** All GUD may present as single or multiple lesions. Syphilis more commonly presents as a single, painless ulcer. A history of recurrence at the same site and/or vesicles or small shallow ulcer(s) suggests HSV. Chancroid, LGV, and GI are rare in Jamaica.
**Step 5:** Treat as indicated and refer all patients with reactive syphilis to the CI for review in 7 days. See the *Practical Case Management of Common STI Syndromes* for clinical details.
Treat for LGV - **Doxycycline 100 mg, orally, twice daily for 21 days**
Treat for Chancroid - **Azithromycin, 1g, in a single dose**
Treat for GI - **Azithromycin, 1g, orally, once weekly for 3 weeks**

**Figure 10: Field Algorithm for Laboratory Diagnostic Testing for Syphilis**

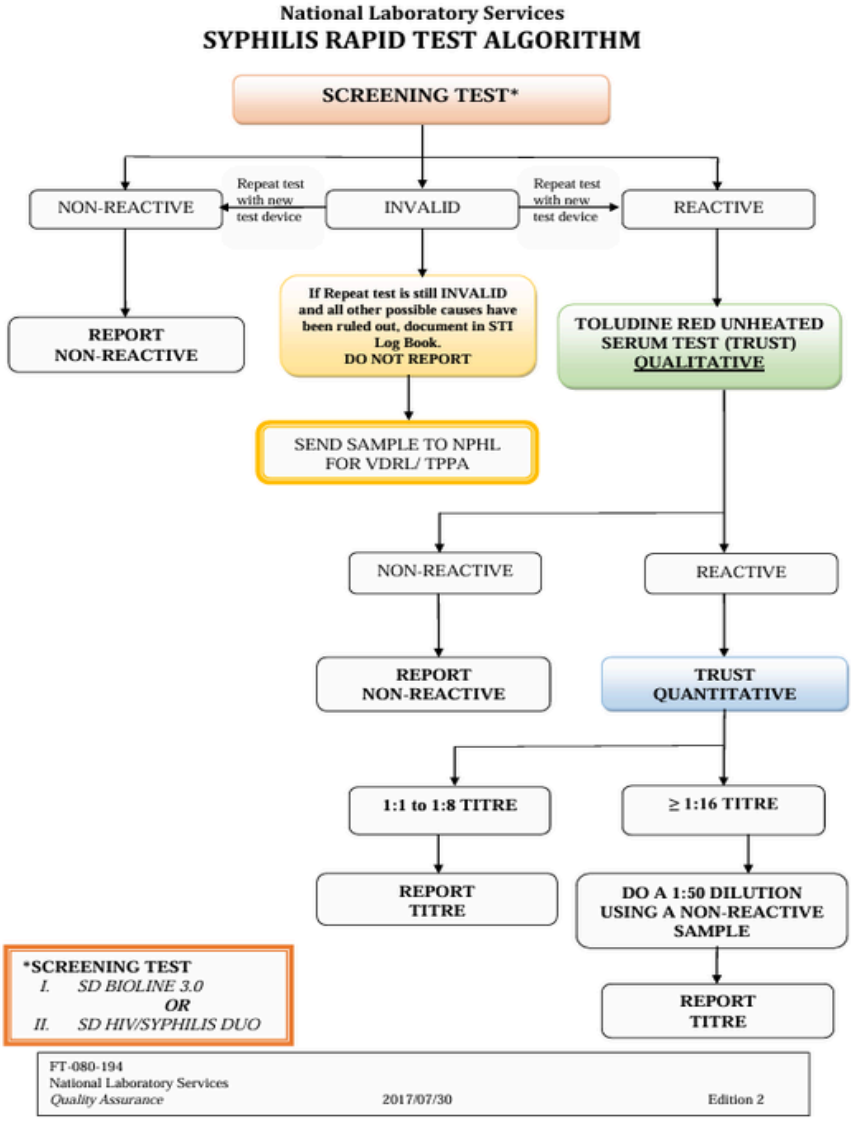


**Table 8: Syphilis Stages and Test Results**

| Stage of Syphilis | Treponemal Test Result e.g. *SD Bioline, TPPA, POC kits | Non-Treponemal Test Result e.g. *VDRL, TRUST |
|---|---|---|
| Primary (sore) (Incubation period: 9-90 days) | Reactive (78 - 86%) | Reactive (76 - 84%), > R1:8 |
| Secondary (rash) (usually occurs 2-8 weeks after primary stage) | Reactive (100%) | Reactive (100%), > R1:8 |
| Early Latent (infection of less than 1 year duration without clinical signs) | Reactive (97 – 100%) | Reactive (95 - 100%), > R1:8 |
| Late Latent (infection of 1 or more years duration without clinical signs) | Reactive (100%) | Reactive, > R1:8 |
| Tertiary | Reactive (94 – 96%) | Reactive (71 - 73%), > R1:8 |

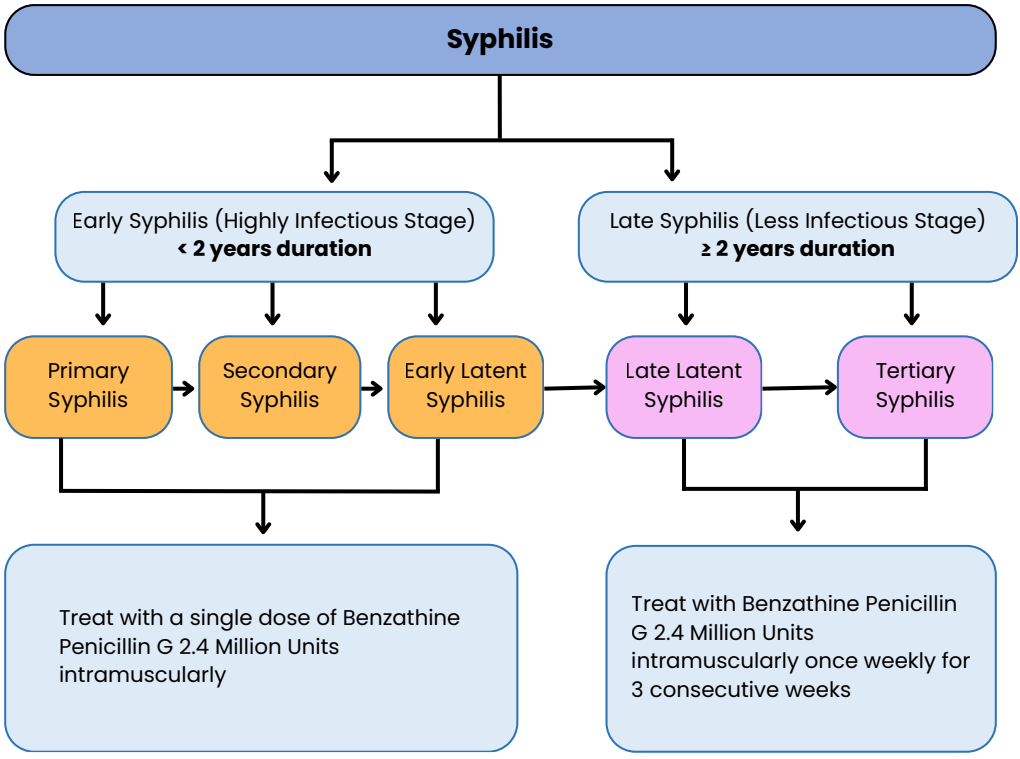
**Notes to table 8:**
*SD Bioline
TPPA-Treponema Pallidum Particle Agglutination Assay
POC-Point-of-Care
VDRL-Venereal Disease Research Laboratory
TRUST-Toluidine Red Unheated Serum Test

**Table 9: Guide to the Interpretation of Syphilis Serologic Test Results**

| Treponemal test result e.g. *SD Bioline, TPPA, POC kits | Non-Treponemal test result e.g. *VDRL, TRUST | Interpretation |
|---|---|---|
| Reactive | Reactive | Syphilis may be treated or untreated. If the initial titre is ≥ 1:8, it is likely indicative of infectious syphilis. However, if the pre-treatment titre was high and there is a documented fourfold decrease in titre following treatment, the patient is likely no longer infectious. |
| Reactive | Non-reactive | This pattern is typically indicative of treated syphilis but may also suggest primary syphilis, very early latent syphilis, or late latent syphilis. Differentiation relies on a detailed history and clinical examination. |
| Non-reactive | Reactive | Biological false positive |

**Notes to table 8:**
*SD Bioline
TPPA-Treponema Pallidum Particle Agglutination Assay
POC-Point-of-Care
VDRL-Venereal Disease Research Laboratory
TRUST-Toluidine Red Unheated Serum Test

**Figure 11: Algorithm for the Management of Syphilis**

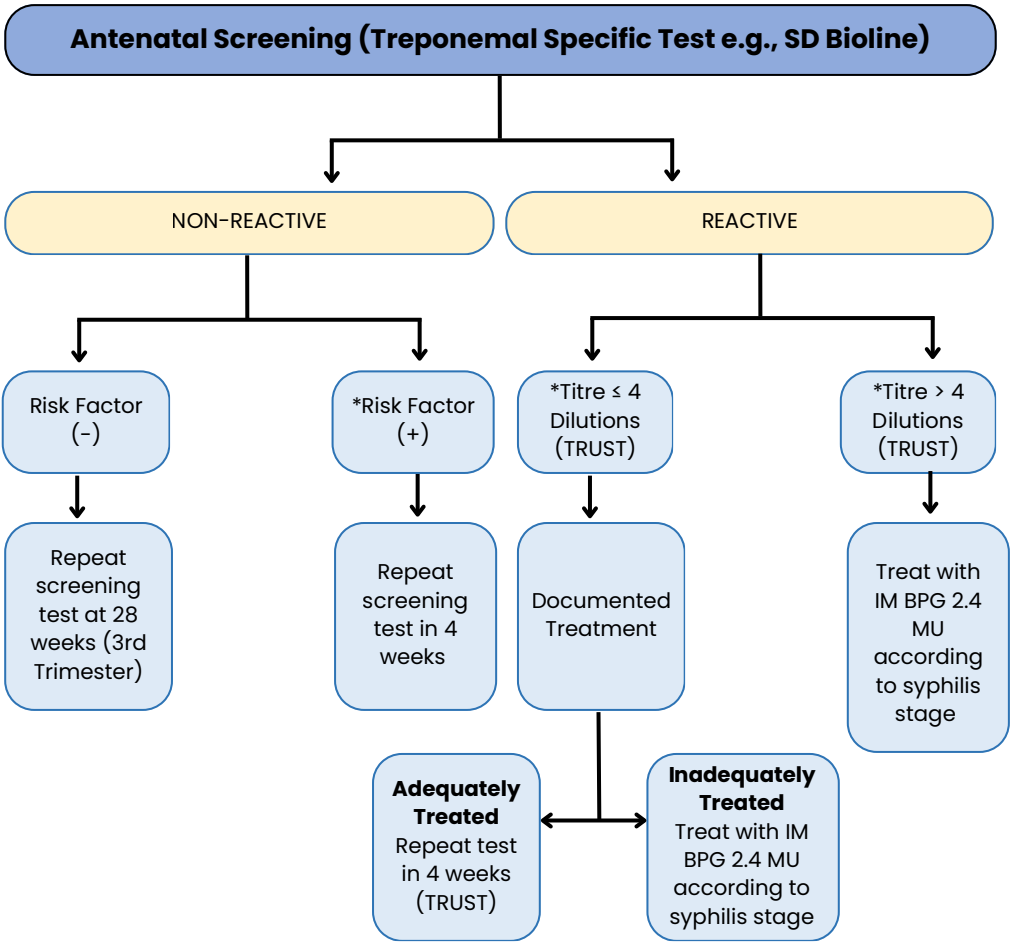



**Note to Figure 11:**
*First-line treatment for tertiary syphilis is recommended for cases without neurological or ocular involvement.

**Table 10: Treatment of Genital Ulcer Disease - Syphilis**

| Aetiology | Treatment |
|---|---|
| **Syphilis**<br><br>(*Primary, secondary, and early latent stages*: these stages are considered highly infectious and occur within two years of infection.) | • Benzathine Penicillin G (BPG) 2.4 million units, intramuscularly in a single dose<br>**OR**<br>• Doxycycline 100 mg, orally, twice daily for 14 days<br>**OR**<br>• Erythromycin 500 mg, orally, four times daily for 14 days |
| **Syphilis**<br><br>(*Late latent and tertiary stages*: untreated individuals in these stages are less likely to transmit the infection as latency progresses and occurs more than two years of infection.) | • Benzathine Penicillin G (BPG) 2.4 million units, intramuscularly once weekly for three (3) consecutive weeks<br>**OR**<br>• Doxycycline 100 mg, orally, twice daily for 30 days<br>**OR**<br>• Erythromycin 500 mg, orally, four times daily for 30 days |

**Figure 12: Algorithm for Management of Maternal Syphilis**



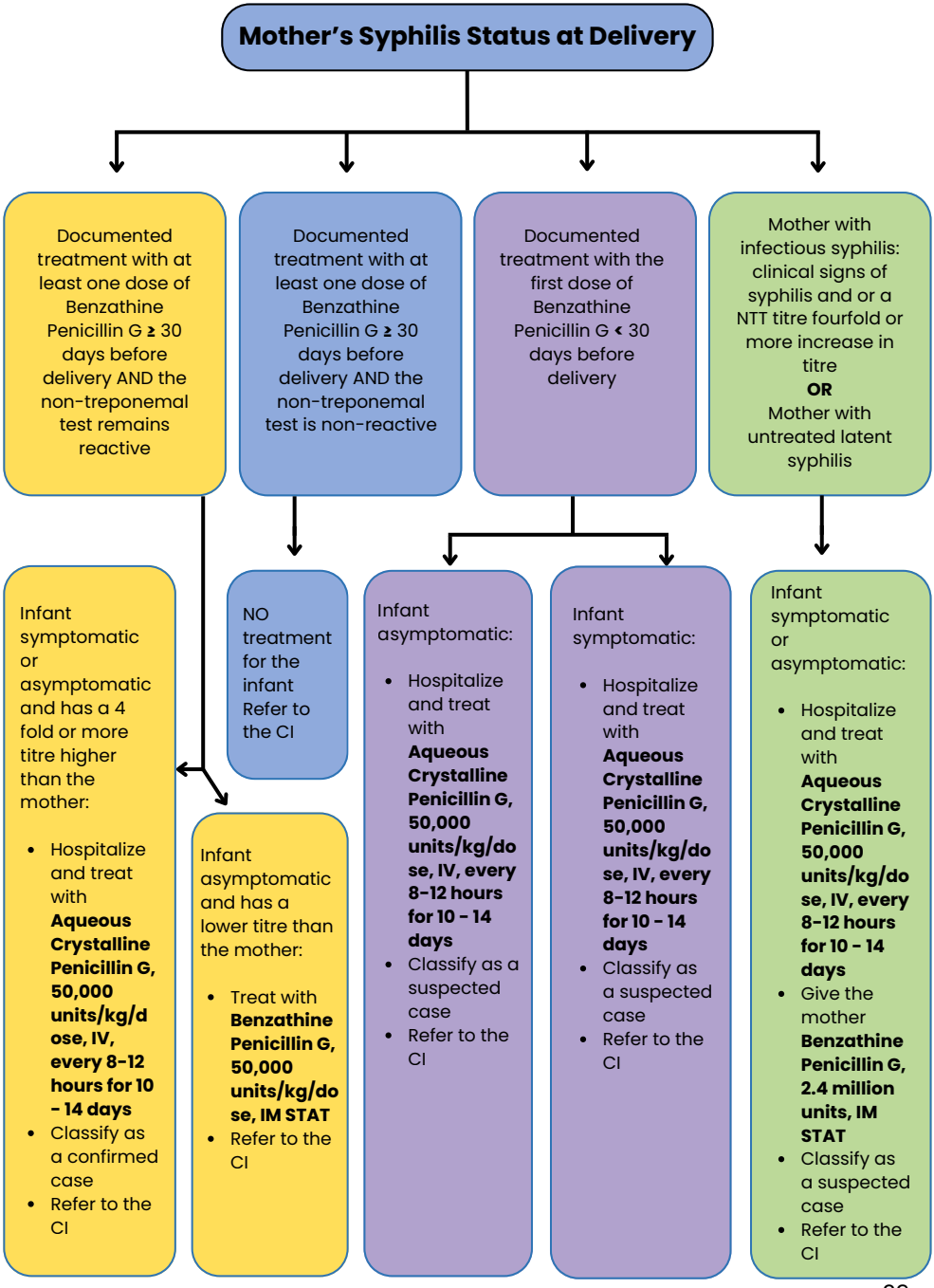

**Note to Figure 12:**

- **Best Practice:** A single intramuscular dose of Benzathine Penicillin G, 2.4 MU, is encouraged for any pregnant woman who tests reactive on the SD Bioline test, even if she is serofast.
- *If the history or clinical assessment indicates a syphilis infection, provide treatment according to the identified stage and refer the case to the Contact Investigator (CI) for partner management.
- All syphilis seropositive mothers allergic to penicillin must be referred to an internist for **penicillin desensitization**. Medications such as erythromycin do not cross the placental barrier and is not adequate to treat the foetus.

## Table 11: Treatment for Maternal Syphilis

| Condition | Recommended Treatment |
|---|---|
| Primary<br>(≤ 2 years) | Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose |
| Secondary and Early Latent<br>(≤ 2 years) | Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose |
| Late Latent and Tertiary<br>(> 2 years) | Benzathine Penicillin G, 2.4 million units, intramuscularly, single dose for 3 consecutive doses one week apart<br><br>(First-line treatment for tertiary syphilis is recommended for cases without neurological or ocular involvement.) |
| Penicillin-Allergic Patients with Syphilis | Refer to internal medicine or an infectious diseases specialist for penicillin desensitisation. |

## Figure 13: Algorithm for the Management of Congenital Syphilis



## Table 12: Treatment for Congenital Syphilis

| Condition | Recommended Treatment |
|---|---|
| Early Congenital Syphilis < 2 years | Aqueous Crystalline Penicillin G (ACPG): 50,000 units/kg/dose IV every 8 – 12 hours (100,000-150,000 units daily) for 10-14 days.<br>**OR**<br>Aqueous Procaine Penicillin G (APPG): 50,000 units/kg IM stat daily for 10-14 days.<br><br>All syphilis seropositive antenatal women must be treated with at least a single dose of Benzathine Penicillin G, 2.4 Million Units, intramuscularly, for **greater than 30 days** before delivery.<br><br>When the mother has been treated with erythromycin during pregnancy or BPG < 30 days before delivery, this is a probable case of congenital syphilis, which requires admission to a secondary care facility for confirmation and appropriate treatment.<br><br>(**N.B.** Erythromycin passes the placental barrier poorly). |

## Table 13: Treatment of Genital Ulcer Disease – Herpes Simplex Virus (HSV)

| Aetiology | Treatment |
|---|---|
| **Herpes Simplex Virus** | Symptomatic primary episode:<br>• Acyclovir 400 mg, orally, 3 times/day for 7 - 10 days<br>**OR**<br>• Acyclovir 200 mg, orally, 5 times/day for 7 - 10 days<br>**OR**<br>• Valacyclovir 500 mg twice daily for 7 - 10 days<br>Recurrent episodes:<br>• Acyclovir 400 mg, orally, 3 times/day for 5 days<br>**OR**<br>• Acyclovir 800 mg, orally, twice daily for 5 days<br>**OR**<br>• Acyclovir 800 mg, orally, 3 times/day for 2 days<br>**OR**<br>• Valacyclovir 500 mg, orally, twice daily for 5 days<br><br>Suppressive therapy:<br>• Acyclovir 400 mg, orally, twice daily for 6 months then reassess (MANAGEMENT MAY EXTEND UP TO A YEAR)<br>**OR**<br>• Valacyclovir 500 mg, orally, once daily for 6 months then reassess (MANAGEMENT MAY EXTEND UP TO A YEAR)<br><br>Plus Saline Sitz Baths as needed |

**Table 14: Treatment of Genital Ulcer Disease – Chancroid, Granuloma Inguinale and Lymphogranuloma Venereum**

| Aetiology | Treatment |
|---|---|
| Chancroid | Azithromycin 1 g, orally, in a single dose<br>**OR**<br>Ceftriaxone 250 mg, intramuscularly, in a single dose<br>**OR**<br>Ciprofloxacin 500 mg, orally, twice daily for three (3) days<br>**OR**<br>Erythromycin 500 mg, orally, 3 times daily for 7 days |
| Granuloma inguinale<br>(Treat until completely cured. Follow up monthly for at least 6 months.) | • Azithromycin 1 g, orally, once weekly for 3 weeks and until all lesions have completely healed<br>**OR**<br>• Azithromycin 500 mg, orally, once weekly for 3 weeks and until all lesions have completely healed.<br>**OR**<br>• Doxycycline 100 mg, orally, twice daily for 3 weeks and until all lesions have completely healed.<br>**OR**<br>• Erythromycin 500 mg, orally, 4 times/day for 3 weeks and until all lesions have completely healed.<br>**OR**<br>• Trimethoprim-sulfamethoxazole (160 mg/800 mg), one double-strength (1DS) tablet, orally, twice daily for 3 weeks and until all lesions have completely healed. |
| Lymphogranuloma venereum | • Doxycycline 100 mg, orally, twice daily for 21 days<br>**OR**<br>• Azithromycin 1 g, orally, once weekly for 3 weeks<br>**OR**<br>• Erythromycin, 500 mg, orally, 4 times/day for 21 days |

## Table 15: Prescribing Pre-Exposure Prophylaxis (PrEP)

| PrEP Method | How is it taken? | Who is it for? | What is it for? |
|---|---|---|---|
| Daily oral PrEP | One pill is taken daily | All populations | Preventing HIV acquisition during all types of exposures |
| Event-driven (ED) PrEP | Two pills are taken 2-24 hours before sex and then a pill each following day until two days after the last potential sexual exposure | People assigned male at birth, not using exogenous hormones, who can plan their sexual activity | Preventing HIV acquisition during SEXUAL EXPOSURES ONLY |

**Notes to table 15:**
PrEP is to be prescribed to people who are HIV negative with a substantial risk of acquiring HIV and have a creatinine clearance of > 60 ml/min.

**Table 16: Antiretroviral Therapy for Adolescents and Adults living with HIV/AIDS**

| **First Line ARVs** | | |
|---|---|---|
| First choice | Tenofovir/Lamivudine (TDF/3TC) | Dolutegravir (DTG) |
| Second choice | Tenofovir/Lamivudine (TDF/3TC) | Efavirenz (EFV) |
| **Second Line ARVs** | | |
| First choice | Abacavir/Lamivudine (ABC/3TC) | Atazanavir/Ritonavir (ATV/r) |
| Second choice | Abacavir/Lamivudine (ABC/3TC) | Lopinavir/Ritonavir (LPV/r) |

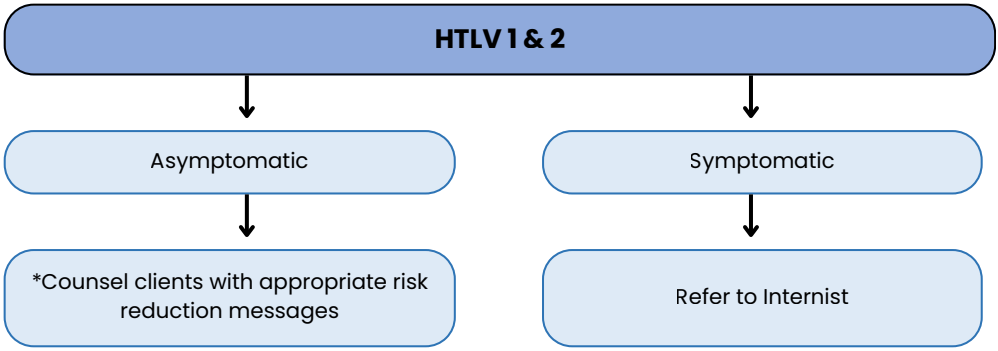
**Table 17: Recommended Post-Exposure Prophylaxis (PEP) following Occupational and Non-Occupational Exposure to a Known or Unknown Source**

| Type of Exposure | Risk | Source | Antiretroviral | Suggested Regimen |
|---|---|---|---|---|
| Percutaneous | High risk | Known HIV positive | Recommended | Tenofovir/Lamivudine /Dolutegravir (TDF/3TC/DTG) Duration 4 weeks |
| | Low risk | Unknown Serostatus | Should be offered (Consider for unknown serostatus) | Tenofovir/Lamivudine /Dolutegravir (TDF/3TC/DTG) Duration 4 weeks |
| Mucous Membranes, Non-intact skin | Large volume | Known HIV positive | Should be offered (Consider for unknown serostatus) | Tenofovir/Lamivudine /Dolutegravir (TDF/3TC/DTG) Duration 4 weeks |
| | Small volume (few drops) | Unknown Serostatus | Should be offered (Not recommended for unknown serostatus) | Tenofovir/Lamivudine /Dolutegravir (TDF/3TC/DTG) Duration 4 weeks |

**Notes for table 17:**

- PEP should be offered ideally within hours of exposure; however, clinical benefit remains up to 72 hours.
- Alternatives for ART choices include:
  - For Tenofovir/Lamivudine (TDF/3TC), the alternative is Abacavir/Lamivudine (ABC/3TC).
  - For Dolutegravir (DTG), the alternative is Atazanavir/Ritonavir (ATV/r).
  - These alternatives, similarly to the recommended option should be given for 4 weeks.

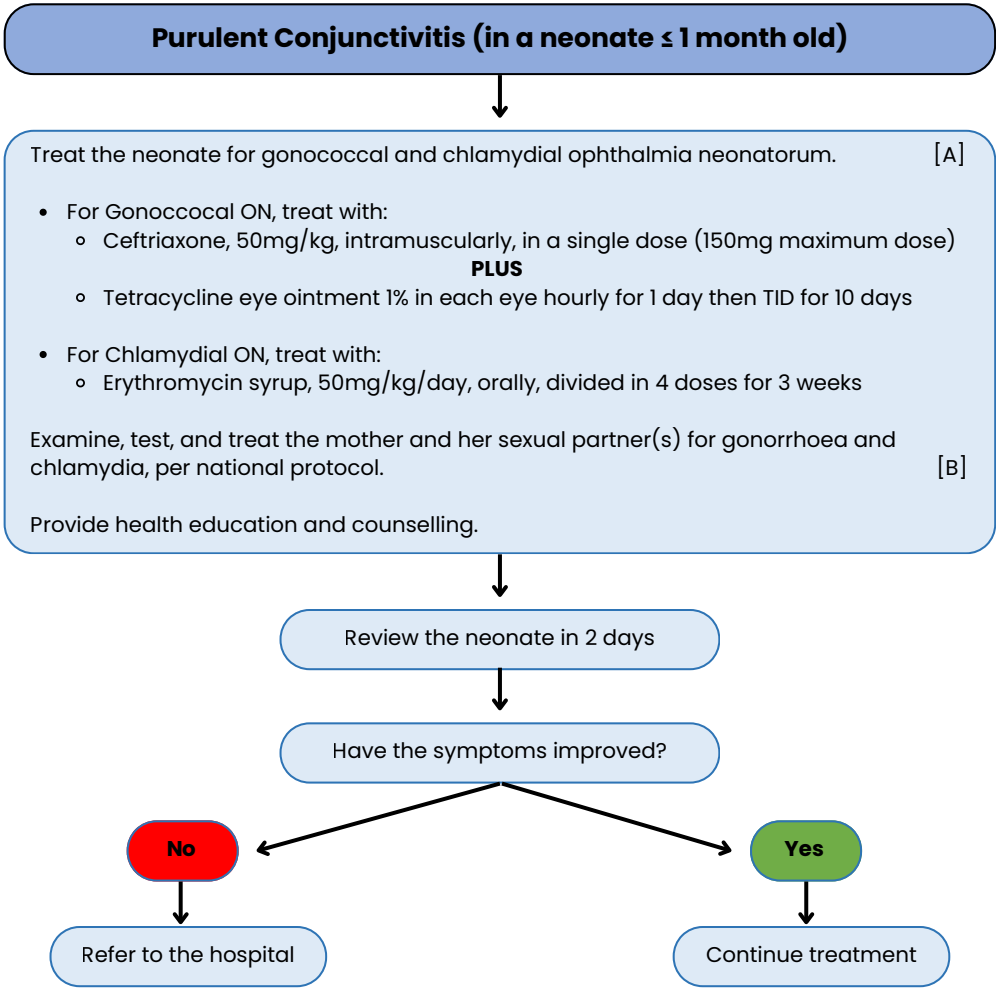
**Figure 14: Algorithm for Management of Human T Lymphotropic Virus Type I/II (HTLV-I/II)**




**Notes to Figure 14:**
Clients who test positive for HTLV I or II should be encouraged to use condoms during sexual activity.
Pregnant women with HTLV should also be encouraged **NOT** to breastfeed.

## Figure 15: Algorithm for Management of Ophthalmia Neonatorum (ON)

(Without Laboratory support)




**Notes to Figure 15:**
ON is a medical emergency and best managed in hospital
[A] Given the intermittent unavailability of erythromycin syrup and its poor activity against N. gonorrhoeae, use tetracycline eye ointment plus systemic therapy active against the N. gonorrhoeae.
[B] In the absence of a confirmed diagnosis, the decision to notify partner(s) should take into account local cultural and epidemiological factors.




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